

12-LEAD ELECTROCARDIOGRAM (ECG) ANALYSIS

Patient Name:	Julianna Sterling	Patient ID:	PT-55219
Age / Gender:	62 Y / Female	Requisition No:	REQ-2026-902
Referred By:	Dr. Robert Chen, MD	Test Date:	06-Jul-2026 09:15 AM
Device Name:	CardioTrack 12X	Status:	Reviewed

ECG QUANTITATIVE METRICS

Parameter	Measured Value	Normal Limits	Interpretation
Heart Rate	104 bpm	60 - 100 bpm	Sinus Tachycardia
PR Interval	156 ms	120 - 200 ms	Normal
QRS Duration	92 ms	80 - 100 ms	Normal
QT / QTc Interval	360 / 448 ms	≤ 460 ms (QTc)	Normal Range
P-R-T Axes	54° / 42° / 38°	-30° to 90°	Normal Axis

ECG WAVEFORM TELEMETRY PREVIEW



[ECG_LEAD_II_STRIP_SNAPSHOT]

Waveform Trace Snippet (Simulated Signal Grid Data)

CLINICAL FINDINGS & MORPHOLOGICAL ANALYSIS

- Rhythm: Regular sinus mechanism pacing at 104 beats per minute.
- P Waves: Normal configuration; 1:1 AV conduction confirmed across all leads.
- ST Segment: Flatline baseline; no acute ST-segment elevation or depression identified to suggest active transmural ischemia or acute coronary event.
- T Waves: Inversion noted in lateral leads V5 and V6, which may imply chronic myocardial strain or left ventricular hypertrophy substrate.

DIAGNOSTIC IMPRESSION

- **Sinus Tachycardia** at 104 bpm.
- **Nonspecific T-wave inversions** in the lateral leads.

Note: Patient was reported as highly anxious during the clinical encounter, which likely accounts for the heart rate acceleration. No classic signs of acute ischemia present on current trace.

Electronically signed off by attending cardiologist.

Dr. Vance Sterling, MD, FACC

Director of Electrocardiography

NPI Number: 1984210943